

CITYCARE GENERAL HOSPITAL

PATIENT TRANSFER GUIDELINES

Intra-Hospital, Inter-Hospital & International Patient Transfer Framework

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Prepared By	Quality Assurance & Clinical Governance Team
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TABLE OF CONTENTS

1	Purpose, Scope & Guiding Principles
2	Definitions & Transfer Classification
3	Roles & Responsibilities
4	General Transfer Safety Standards
5	Intra-Hospital Transfer Workflow
6	Inter-Hospital Transfer — Outgoing Patients
7	Inter-Hospital Transfer — Incoming Patients
8	Critical Care & High-Dependency Transfers
9	Paediatric, Obstetric & Vulnerable Patient Transfers
10	Documentation, Exceptions & Best Practices

1. PURPOSE, SCOPE & GUIDING PRINCIPLES

The safe transfer of patients between wards, departments, hospitals, and care settings is a high-risk clinical activity. Transfer-related adverse events, including clinical deterioration, equipment failure, communication breakdown, and documentation gaps, are among the most preventable causes of patient harm. This policy establishes mandatory safety standards and structured workflows for all patient transfers undertaken at, from, or to CityHospital, ensuring every transfer is clinically justified, properly prepared, and safely executed.

1.1 Policy Objectives

- Define minimum safety standards for all patient transfers, regardless of urgency or destination.
- Ensure every transfer is clinically justified and the risk-benefit is clearly documented.
- Standardise the pre-transfer assessment, clinical handover, and documentation processes.
- Mandate appropriate escort qualifications matched to the clinical acuity of the patient.
- Ensure continuity of care and seamless information transfer at every handover point.
- Prevent adverse events during transfer through proactive equipment checks and monitoring.
- Comply with national patient transport regulations, ambulance service protocols, and accreditation requirements.

1.2 Scope

- **Intra-hospital transfers:** Movement of a patient between any two clinical areas within CityHospital (ward-to-ward, ward-to-theatre, ED-to-ICU, ICU-to-ward).
- **Inter-hospital transfers:** Movement of a patient between CityHospital and another healthcare facility, including referral centres, rehabilitation units, nursing homes, or private hospitals.
- **International / repatriation transfers:** Transfer of patients to or from overseas facilities; governed additionally by international medical transport standards.

NOTE: This policy applies to all clinical staff involved in patient transfers. Agency, locum, and contracted staff must be made aware of this policy at induction. Non-compliance is a patient safety violation subject to clinical governance review.

2. DEFINITIONS & TRANSFER CLASSIFICATION

Term / Abbreviation	Definition
Intra-Hospital Transfer	Transfer between two clinical areas within the same hospital building or campus.
Inter-Hospital Transfer	Transfer between CityHospital and a separate healthcare facility.
Primary Transfer	Transfer of a patient directly from the scene of an emergency or from a first point of contact to a definitive care facility.
Secondary Transfer	Transfer of an already hospitalised patient from one facility to another for definitive or specialised care.
Repatriation Transfer	Transfer of a patient back to their local hospital or country after receiving specialist care elsewhere.
Accompanied Transfer	Transfer with a qualified clinical escort (nurse, paramedic, or physician) throughout the journey.
Unaccompanied Transfer	Transfer without a clinical escort; only appropriate for ambulatory, clinically stable patients with no monitoring requirements.
SBAR	Situation–Background–Assessment–Recommendation — structured verbal handover framework used at every clinical transfer.
Receiving Clinician	The physician or senior nurse accepting clinical responsibility for the patient at the destination.
Transfer Risk Score	A standardised assessment score (e.g., CCMU or NACA) used to determine appropriate escort level and transport mode.
Critical Transfer	Transfer of a patient requiring continuous monitoring, active resuscitation support, or ongoing organ support during transit.
MCA	Mental Capacity Act — governs consent to transfer for patients who may lack decision-making capacity.
NOK	Next of Kin — patient's designated family member or legal guardian; must be notified of all inter-hospital transfers.

3. ROLES & RESPONSIBILITIES

Role	Department	Transfer Responsibilities
Referring / Transferring Physician	Any Clinical Dept.	Clinically justify transfer; obtain consent; complete documentation; provide SBAR handover; remain contactable
Receiving Physician / Consultant	Destination Unit	Formally accept transfer in writing; confirm bed availability; be available for questions during transit
Transfer Co-ordinating Nurse	Ward / ICU / ED	Organise transfer logistics; complete transfer checklist; escort if required; handover at destination
ICU Consultant	ICU / Critical Care	Authorise all critical care transfers; determine escort level; brief transfer team
Bed Manager	Bed Management	Confirm bed availability at destination (intra-hospital); coordinate with receiving hospital for inter-hospital
Portering Team	Operations	Intra-hospital patient transport; equipment management during transfer
Ambulance / Transport Service	External / Patient Transport	Provide safe vehicle and crew; follow clinical handover; alert receiving team on arrival
Patient Relations / Social Work	Support Services	Notify NOK; arrange family support; assist with consent for incapacitated patients
Site Manager / Hospital at Night	Site Management	Authorise out-of-hours critical transfers; escalate issues to CMO if required

4. GENERAL TRANSFER SAFETY STANDARDS

4.1 Pre-Transfer Clinical Assessment

- Every patient must be clinically assessed by the transferring physician immediately before transfer; assessment documented in EMR.
- Vital signs (BP, HR, RR, SpO₂, GCS, temperature) recorded within 30 minutes of transfer commencing.
- Transfer risk score calculated; escort level determined using the table in Section 4.3.
- Active clinical problems stabilised before transfer unless transfer is itself part of the treatment (e.g., transfer for emergency surgery).
- IV access patent and secured; infusions labelled and flow rates set for transit duration.
- Airway management plan documented for all patients with compromised or potentially compromised airways.

4.2 Patient Consent for Transfer

- Patients with full mental capacity must receive a clear explanation of the reason for transfer, destination, and expected care; verbal consent documented in EMR.
- For patients lacking capacity (MCA assessment documented): transfer decision made in the patient's best interest; NOK informed; two-physician sign-off for inter-hospital transfers.
- Patients who refuse transfer: full clinical risk explained; refusal documented; patient signs transfer refusal form; duty of care maintained in current facility.

4.3 Escort Level Classification

Escort Level	Patient Criteria	Minimum Escort Required
Level 0 — Unescorted	Fully ambulatory, clinically stable, no monitoring, no IV access	None — patient travels independently or with family
Level 1 — Porter Escort	Stable, minimal monitoring (SpO ₂ only), low falls risk	Trained hospital porter; nurse available at destination
Level 2 — Nurse Escort	IV infusions running, moderate acuity, SpO ₂ and BP monitoring	Registered nurse (RN); resus bag and portable monitor
Level 3 — Senior Nurse / Paramedic	Single-organ support, haemodynamically labile, oxygen-dependent	Senior RN or paramedic; full portable monitoring; drug kit
Level 4 — Physician Escort	Mechanical ventilation, active resuscitation, multi-organ support	ICU physician or anaesthetist; full transfer ventilator; emergency drugs

5. INTRA-HOSPITAL TRANSFER WORKFLOW

5.1 Standard Intra-Hospital Transfer

- Step 1.** Transferring physician enters transfer order in EMR, specifying destination, clinical reason, escort level, and any special requirements (oxygen, monitoring, isolation).
- Step 2.** Transferring nurse completes the Intra-Hospital Transfer Checklist (see Section 10.1); confirms all items ticked before patient leaves the ward.
- Step 3.** Bed Manager notified via BMS; receiving ward confirms bed is ready and staff are expecting the patient.
- Step 4.** Receiving ward nurse briefed on patient condition, active clinical issues, and any pending actions.
- Step 5.** Porter or nurse escort collects patient; portable monitoring applied as per escort level.
- Step 6.** SBAR verbal handover delivered by transferring nurse to receiving nurse at the bedside of the new location.
- Step 7.** Receiving nurse signs transfer checklist; assumes clinical responsibility; documents patient arrival in EMR.
- Step 8.** Transferring ward marks bed as 'vacated' in BMS immediately; bed turnaround process initiated.

5.2 Emergency Intra-Hospital Transfer (e.g., Deteriorating Patient)

- Step 1.** Clinical deterioration identified; Medical Emergency Team (MET) or Rapid Response Team activated if indicated.
- Step 2.** Physician assesses and determines urgent transfer requirement (e.g., ward to ICU).
- Step 3.** ICU/HDU bed confirmed immediately by Bed Manager; ICU team prepared to receive.
- Step 4.** Patient transferred with highest appropriate escort level; do not wait for paperwork — clinical safety takes priority.
- Step 5.** Full SBAR handover at ICU bedside; documentation completed within 1 hour of arrival.

NOTE: For any intra-hospital transfer of a Level 3 or Level 4 escort patient, the transferring physician must personally brief the escort and must be immediately contactable by phone throughout the duration of the transfer.

6. INTER-HOSPITAL TRANSFER — OUTGOING PATIENTS

6.1 Indications for Inter-Hospital Transfer

- Patient requires specialist investigation, intervention, or surgical expertise not available at CityHospital.
- Patient requires a higher or lower level of care available only at a different facility (e.g., specialist ICU, burns unit, cardiac surgery centre).
- Capacity-driven transfer during surge (Level 4 occupancy) — only if clinically safe and patient consents.
- Repatriation to patient's local hospital following treatment at CityHospital.
- Patient or family request transfer to a specific facility — clinically assessed as safe before agreed.

6.2 Outgoing Inter-Hospital Transfer Workflow

Step 1. Referring consultant identifies need for inter-hospital transfer; documents clinical justification in EMR.

Step 2. Receiving hospital consultant contacted directly by referring consultant; verbal acceptance confirmed; written acceptance in EMR within 2 hours.

Step 3. Bed confirmed at receiving facility before transfer is arranged; written bed confirmation sent to Bed Manager.

Step 4. Patient and NOK informed of transfer; reason, destination, expected timeline, and contact details provided.

Step 5. Transfer risk score calculated; appropriate transport mode selected (hospital car, non-emergency ambulance, critical care ambulance).

Step 6. Transferring nurse prepares comprehensive transfer documentation pack (see Section 10.2).

Step 7. Transfer checklist completed and signed by transferring physician and nurse before patient leaves.

Step 8. Ambulance crew given verbal SBAR handover and full written documentation; handover time logged in EMR.

Step 9. Receiving hospital notified of departure time and estimated arrival; alerts them to any active clinical concerns.

Step 10. Transferring physician remains contactable until receiving physician confirms patient arrived safely and accepted care.

WARNING: An inter-hospital transfer must NEVER be initiated without confirmed acceptance by a named receiving clinician and confirmed bed availability. Transferring a patient without receiving hospital acceptance is a serious patient safety violation.

7. INTER-HOSPITAL TRANSFER — INCOMING PATIENTS

7.1 Accepting an Incoming Transfer

- Step 1.** Receiving consultant at CityHospital accepts the referral verbally; documents clinical basis for acceptance in EMR.
- Step 2.** Bed Manager confirms appropriate bed type and location; informs Ward Manager and nursing team of incoming patient.
- Step 3.** Estimated arrival time communicated to ED (if arriving via ED) or directly to the receiving ward.
- Step 4.** Receiving ward prepares bay or room: appropriate monitoring, IV access materials, oxygen, and specialist equipment as briefed.
- Step 5.** Infection control requirements communicated to Ward Manager if transferring hospital has flagged any MDRO or isolation needs.
- Step 6.** Receiving nurse assigned as named clinical contact; briefed on expected clinical condition and active clinical issues.

7.2 Receiving the Incoming Patient

- Step 1.** Ambulance crew or escort delivers patient; verbal SBAR handover provided to receiving nurse at bedside.
- Step 2.** Receiving nurse performs immediate clinical assessment: vital signs, pain score, IV access, medication infusions.
- Step 3.** Transfer documentation reviewed: discharge summary, drug chart, investigation results, imaging, consent forms.
- Step 4.** Any clinical concerns or discrepancies between verbal handover and clinical state reported to receiving consultant immediately.
- Step 5.** Patient identity verified using 3-point check (name, DOB, MRN); new identity wristband applied.
- Step 6.** Transfer documentation scanned and linked to patient MRN in EMR within 2 hours of arrival.
- Step 7.** Patient and NOK welcomed; ward orientation provided; next of kin contact number confirmed.

7.3 Infectious Patient Arriving from Another Facility

- Sending hospital must declare all known or suspected MDROs, active infections, or isolation requirements before transfer.
- IPC Nurse notified of arriving infectious patient before ambulance departs; isolation room confirmed and prepared.
- Ambulance crew briefed on appropriate PPE on arrival; patient transferred directly to isolation room on arrival — no transit through open ward areas.
- Microbiological screening swabs taken on arrival as per IPC protocol.

8. CRITICAL CARE & HIGH-DEPENDENCY TRANSFERS

Transfers involving critically ill patients carry the highest risk and require the most rigorous preparation. The following standards are mandatory for all Level 3 and Level 4 escort transfers, whether intra- or inter-hospital.

8.1 Pre-Transfer Stabilisation Checklist — Critical Patients

- Airway secured and patent; endotracheal tube position confirmed on CXR within 2 hours of transfer.
- Ventilator settings documented; transfer ventilator checked and functioning; adequate oxygen cylinder capacity confirmed for journey + 30-minute contingency.
- Haemodynamic stability: MAP above 65 mmHg maintained; vasopressor infusions prepared for transfer duration with spare syringes.
- Sedation and analgesia infusions labelled, capped, and set for transfer; spare ampoules packed.
- All IV lines patent, secured, and labelled; central line dressings intact.
- Portable defibrillator charged and immediately accessible; resuscitation drugs drawn up.
- Full set of vital signs documented within 15 minutes of transfer departure.
- Receiving ICU formally briefed on ventilator settings, vasopressor requirements, active problems, and latest blood results.

8.2 During Transfer — Critical Patient Monitoring

Parameter	Monitoring Requirement	Action if Abnormal
SpO2	Continuous pulse oximetry throughout transfer	Increase FiO2; check airway; notify physician immediately
ECG	Continuous 3-lead monitoring	Interpret rhythm; administer antiarrhythmic if trained; halt transfer if unstable
NIBP / ABP	Every 5 minutes (NIBP) or continuous (ABP)	Titrate vasopressors; fluid bolus if indicated; notify physician
ETCO2	Continuous capnography for all ventilated patients	Adjust ventilator; check circuit; re-intubate if tube displacement
Infusion pumps	Visual check every 5 minutes	Ensure batteries charged; carry backup syringe drivers

8.3 Transfer Interruption Protocol

- If the patient deteriorates en route: halt transfer in a safe location; initiate resuscitation as per ABCDE protocol.
- For intra-hospital transfers: return to nearest clinical area with full resuscitation facilities.
- For inter-hospital transfers: divert to nearest emergency department; notify both sending and receiving hospitals immediately.
- Post-transfer incident: complete clinical incident form within 24 hours; debrief transfer team.

9. PAEDIATRIC, OBSTETRIC & VULNERABLE PATIENT TRANSFERS

9.1 Paediatric Transfers

- All paediatric inter-hospital transfers (under 16 years) require consultant paediatrician authorisation.
- Weight-based drug preparation mandatory for all paediatric transfers; Broselow tape or EMR paediatric dosing tool used.
- Parent or guardian must accompany child on all non-critical intra-hospital transfers; for critical inter-hospital transfers, parent travels in ambulance where space and safety permit.
- Paediatric transfer equipment bag checked and sealed before every transfer; contents per paediatric resuscitation standard.
- Neonatal transfers to specialist neonatal units: neonatal transport incubator used; neonatologist or specialist transport team escort mandatory.
- Safeguarding concern during transfer: social worker notified; child does not travel with the individual raising the concern.

9.2 Obstetric Transfers

- Obstetric transfers require obstetric and neonatal team assessment before departure; both maternal and fetal wellbeing confirmed.
- No obstetric patient in active labour (cervical dilatation above 4 cm) transferred by road unless delivering en route is clinically safer than staying.
- Imminent delivery risk: midwife escort with delivery pack mandatory for any transfer of a patient in active labour.
- Pre-term labour transfers: receiving neonatal unit bed confirmed before maternal departure; gestation and estimated delivery confirmed with receiving team.

9.3 Patients Lacking Mental Capacity

- Mental Capacity Act (MCA) assessment documented before any inter-hospital transfer of a patient who may lack capacity.
- Best interest decision completed; next of kin or lasting power of attorney consulted and documented.
- Independent Mental Capacity Advocate (IMCA) involved if patient has no appropriate next of kin and transfer is to a different region.
- If patient actively resists transfer: senior physician assessment; force must not be used unless under specific legal authority (e.g., Mental Health Act).

9.4 End-of-Life Transfers

- Transfers of patients in the last days of life must prioritise the patient's expressed wishes for place of care and death.
- Palliative care team involved in transfer planning; symptom management plan documented for transit.
- Receiving hospice, care home, or family home briefed on comfort measures and active management plans.
- Do Not Attempt Resuscitation (DNAR) form, Advance Care Plan, and medication for symptom control travel with the patient.

10. DOCUMENTATION, EXCEPTIONS & BEST PRACTICES

10.1 Intra-Hospital Transfer Checklist

- Patient identity band checked and correct.
- Transfer order signed by physician in EMR.
- Vital signs recorded within 30 minutes; abnormal values reviewed by physician.
- Active medications reviewed; IV infusions labelled and running at correct rate.
- Relevant investigation results and imaging available at destination.
- Receiving ward nurse briefed verbally by transferring nurse (SBAR).
- Escort level confirmed and appropriate personnel assigned.
- Portable monitoring applied as per escort level.
- Patient's personal belongings, valuables, and dentures logged and transferred with patient.
- Transfer documented in EMR; BMS updated to reflect bed vacated.

10.2 Inter-Hospital Transfer Documentation Pack

Document	Prepared By	Mandatory For
Transfer summary / referral letter	Referring physician	All inter-hospital transfers
Current medication chart (full)	Ward pharmacist / nurse	All inter-hospital transfers
Recent investigation results (last 48 hrs)	Ward nurse / physician	All inter-hospital transfers
Imaging — printed or digital CD	Radiology	Surgical, neurological, trauma transfers
Consent forms (surgical, anaesthetic)	Physician	Pre-operative transfers
Infection screening results (MRSA, ESBL)	IPC / Microbiology	All inter-hospital transfers
DNAR / Advance Care Plan	Physician / Palliative team	End-of-life and DNR patients
Completed transfer checklist	Transferring nurse + physician	All inter-hospital transfers
NOK contact and notification record	Ward nurse / social work	All inter-hospital transfers

10.3 Exceptions

- Life-threatening emergencies (cardiac arrest en route, acute airway loss): immediate intervention takes priority; documentation completed retrospectively within 1 hour.
- Patient refusing transfer with capacity: full risk counselling documented; patient signs refusal form; clinical care continues in current facility.
- Transfer cancelled after departure: receiving hospital notified immediately; patient returned to or stabilised at nearest appropriate facility; incident logged in EMR.

10.4 Best Practices

- Use the structured SBAR framework for every clinical handover at transfer; never rely on documentation alone.

- Conduct a formal transfer briefing with the full escort team before departure for all Level 3 and Level 4 transfers — allocate roles, review clinical condition, and confirm contingency plan.
- Never rush a transfer: a 30-minute delay to properly stabilise a patient is always preferable to a rapid departure with an unstable patient.
- Post-transfer follow-up: for inter-hospital transfers, the referring physician contacts the receiving team within 24 hours to confirm safe arrival and ongoing care.
- Annual transfer skills training for all registered nursing staff: portable monitoring, infusion pump management, and deterioration response during transfer.
- Transfer incident debrief: all transfers involving a clinical incident reviewed at departmental governance meeting within 2 weeks.

10.5 Audit & Quality Indicators

KPI	Target	Frequency
Inter-hospital transfers with confirmed receiving bed before departure	100%	Monthly
Transfer documentation pack complete at departure	100%	Monthly audit sample
Escort level appropriately matched to patient acuity	100%	Monthly audit sample
SBAR handover documented at point of handover	Greater than 95%	Monthly
Transfer-related adverse events	Zero (tracked)	Per event + monthly
NOK notified before inter-hospital transfer	100%	Monthly audit sample